



Havana Specialist Hospital, Board and Committee Instruments Pack

The template suite the board and committees run on

Prepared for: The Board of Havana Specialist Hospital Limited, and the Company Secretary

Prepared by: Dr Debo Odulana, Independent Non-Executive Director; Consult for Africa

Companion to: the Board Operating Model, the Committee Terms of Reference, the Delegation of Authority schedule, and the Board and Committee Handbook

Date: 16 August 2026

Status: For adoption at the September 2026 board meeting

Classification: Confidential

This is Workstream 1 of the enablement engagement: the working instruments. The operating model said how the board should run. The training taught people to run it. This pack is what they actually pick up and use. Every template here derives from the model already adopted, so nothing in it is generic. It is written to be handed to the Company Secretary and administered from the September meeting onward.

1. How to use this pack

Eleven instruments, each self-contained. Together they cover a full governance cycle from the first committee meeting to the minuted board decision.

#	Instrument	Owner	Used when
1	The consolidated board pack	Company Secretary	Every board meeting, circulated 7 days ahead
2	Board and committee agenda	Company Secretary with the Chair	Every meeting
3	Board paper, the one-page decision paper	The sponsoring executive	Any item asking the board to decide
4	Committee report to the board	Each committee chair	Every board meeting
5	Management report by exception	Medical Director and Afya leads	Every board meeting
6	Performance dashboard and KPI pack	Head of Finance and Hospital Manager	Monthly, consolidated quarterly
7	Risk register summary	Audit and Risk chair	Every board meeting
8	Minutes	Company Secretary	Every meeting
9	Action and decision log	Company Secretary	Rolling, carried into every meeting



#	Instrument	Owner	Used when
10	Escalation thresholds schedule	Board, administered by the Company Secretary	Continuously, between meetings
11	Register of interests and attendance	Company Secretary	Maintained continuously, reviewed annually

Plus the **annual board and committee calendar** at section 13.

Two rules that make the suite work. First, nothing reaches the board except through one of these instruments. A verbal update with no paper is not a board item. Second, every template asks the same four things at the end: what is the position against target, what is the exception, what is the risk, and what decision is requested. Once people learn that shape, they can fill in any instrument in the pack.

2. Instrument 1: the consolidated board pack

One pack, one file, assembled by the Company Secretary and circulated **seven clear days** before the meeting. Directors who receive papers late are entitled to say so, and the minute should record it.

2.1 Standing structure

Tab	Section	Prepared by	Length
A	Agenda and meeting notice	Company Secretary with the Chair	1 page
B	Chair's introduction and the three things that matter this quarter	Board Chair	1 page
C	Minutes of the previous meeting, for approval	Company Secretary	as required
D	Action and decision log, with status	Company Secretary	1 to 2 pages
E	Management report by exception	Medical Director and Afya leads	3 pages maximum
F	Performance dashboard and KPI pack	Head of Finance, Hospital Manager	2 pages
G	Finance pack: management accounts, cash, debtors, budget variance	Head of Finance	4 pages maximum
H	Committee reports, one page each from the five committees	Committee chairs	5 pages
I	Afya-HSH Governance Committee report	Joint committee, countersigned by the HSH executive	1 to 2 pages
J	Risk register summary, top risks and movements	Audit and Risk chair	1 to 2 pages



Tab	Section	Prepared by	Length
K	Board papers for decision, one per matter	Sponsoring executive	2 pages each
L	Papers for noting	Various	as required
M	Governance and compliance: statutory filings, register of interests, attendance	Company Secretary	1 page

2.2 Rules for the pack

- **Cap it.** Target 40 pages. If the pack exceeds 60, the problem is the reporting, not the printer.
- **Decision-ready.** Every item is labelled in the agenda as **Decision**, **Discussion**, or **Noting**. A director should be able to tell from the agenda alone what is being asked of them.
- **Exception only.** Sections E to J report variance and exception. Full underlying data belongs in the committees, not the board pack.
- **No tabling on the day.** Papers not circulated with the pack are not taken, unless the Chair rules the matter urgent and the minute records why.
- **One version.** The Company Secretary issues the pack. Directors do not receive separate side-papers from individual executives.

3. Instrument 2: agenda template

Used for the board and, with the committee's own remit substituted, for every committee.



HAVANA SPECIALIST HOSPITAL LIMITED
[BOARD / COMMITTEE NAME] MEETING

Date: [day, date]
Time: [start] to [end]
Venue: [venue, and video link]
Chair: [name]
Secretary: [name]
Quorum required: [number, per the articles or the terms of reference]

No.	Item	Lead	Type	Time
1.	Welcome, quorum, and apologies	Chair	Noting	5 min
2.	Declarations of interest	Chair	Noting	5 min
3.	Minutes of the meeting of [date]	Chair	Decision	5 min
4.	Matters arising and action log	Sec	Noting	10 min
5.	Chair's introduction	Chair	Noting	5 min
6.	Management report by exception	MD	Discussion	20 min
7.	Finance and performance	HoF	Discussion	20 min
8.	Committee reports			
	8.1 Finance, Investment and Operations	Co-chairs	Noting	10 min
	8.2 Audit and Risk	Chair	Noting	10 min
	8.3 Quality and Clinical Governance	Chair	Noting	10 min
	8.4 People and Remuneration	Chair	Noting	5 min
	8.5 Transformation and Digital	Chair	Noting	5 min
9.	Afya partnership performance (standing)	Chair	Discussion	20 min
10.	Risk register summary	ARC chair	Discussion	10 min
11.	Decision items			
	11.1 [matter]	[sponsor]	Decision	15 min
12.	Items for noting	Sec	Noting	5 min
13.	Any other business notified to the Chair	Chair	Noting	5 min
14.	In-camera session, non-executives only	Chair	Discussion	10 min
15.	Date of next meeting	Chair	Noting	2 min

Notes on the agenda.

- **Item 2 is never a formality.** The Chair asks explicitly, and the Company Secretary records each declaration in the minute and in the register.
- **Item 9 is a standing item and cannot be dropped.** It is the board's independent line of sight over the operating partner. If there is nothing to report, the minute says so.
- **Item 14, the in-camera session,** is the non-executives alone, with no executives and no Afya present, for ten minutes at the end of every meeting. It is not an accusation, it is standard practice, and it should be routine from the first meeting so that it never looks like an event.
- **Times are real.** The Chair holds them. A meeting that habitually overruns is an agenda problem, not a discipline problem.

4. Instrument 3: board paper, the one-page decision paper

The instrument that most improves a board. No matter comes to the board for decision without one.



HAVANA SPECIALIST HOSPITAL LIMITED BOARD PAPER

Paper number: [BD/2026/09/01]
 Meeting and date: [Board, 24 September 2026]
 Title: [short, plain title]
 Sponsor: [executive who owns this]
 Author: [if different]
 Committee route: [which committee reviewed it, and what it recommended]
 Type: DECISION / DISCUSSION / NOTING

1. THE DECISION REQUESTED
One sentence. "The board is asked to approve X, at a cost of Y, funded from Z." If you cannot write this sentence, the paper is not ready.
2. WHY NOW
Two or three sentences. What has changed, and what happens if the board does not decide at this meeting.
3. BACKGROUND
Short. Assume directors have read the last pack, not the last decade.
4. THE OPTIONS CONSIDERED
Option A: [option] - cost, benefit, risk
Option B: [option] - cost, benefit, risk
Option C: do nothing - consequence
5. RECOMMENDATION AND WHY
Which option, and the reasoning in plain terms.
6. FINANCIAL EFFECT
Capital and revenue effect, budget line, whether budgeted or not, payback or return where relevant, cash effect and timing.
7. RISKS AND MITIGATIONS
The three that matter, and who owns each.
8. LEGAL, REGULATORY, AND CLINICAL IMPLICATIONS
Including whether this is a reserved matter, whether it touches the Afya agreement, and whether any related party is involved.
9. WHO IS AFFECTED, AND WHO HAS BEEN CONSULTED
Including staff, clinicians, and where relevant both shareholder families.
10. HOW WE WILL KNOW IT WORKED
The measure, the target, and the date it comes back to the board.

Prepared by: [name, role, date]

Section 10 is the one most often omitted and the one that most changes behaviour. A board that always asks "how will we know it worked, and when does it come back" gets better proposals within two cycles.

5. Instrument 4: committee report to the board

One page. Same shape for all five committees and for the Afya-HSH Governance Committee, so the board reads six reports in one familiar format.



COMMITTEE REPORT TO THE BOARD

Committee: [name]
 Chair: [name]
 Meeting date: [date] Attendance: [x of y] Quorate: Yes / No
 Reporting to: Board meeting of [date]

1. WHAT WE DECIDED (within our delegated authority)
 -
 -
2. WHAT WE ARE ASSURED ON
 The two or three things the committee has satisfied itself about, and on what evidence.
3. WHAT WE ARE NOT YET ASSURED ON
 Say it plainly. This section is the reason the committee exists. "Nothing" is an acceptable answer once, and a warning sign twice.
4. EXCEPTIONS AND VARIANCES WE EXAMINED
 Metric, plan, actual, explanation, action, owner, date.
5. RISKS WE ARE ESCALATING TO THE BOARD
 Risk, rating, movement since last quarter, owner.
6. WHAT WE NEED FROM THE BOARD
 DECISION: [specific ask, cross-referenced to board paper number]
 STEER: [where the committee wants the board's direction]
 NOTING: [everything else]
7. OUR NEXT MEETING AND WHAT IS ON IT

Signed: [chair]

Date:

5.1 What each committee reports on

The format is common; the content is not. Each committee brings its own standing set.

Committee	Its standing content in sections 2 to 5
Finance, Investment and Operations	Management accounts against budget, cash and working capital, debtor and HMO receivable position, capital and investment appraisals, operational KPIs and delivery, the commercial performance of the Afya arrangement
Audit and Risk	Internal control findings, internal and external audit progress and findings, the enterprise risk register and its movements, related-party transactions, statutory filings and compliance, going-concern judgement, the independent read on Afya's financial delivery
Quality and Clinical Governance	Clinical quality and safety metrics, serious incidents and never events with the learning from each, mortality and morbidity review, infection prevention, complaints and patient experience, credentialing currency, accreditation and regulatory readiness, clinical risk feeding the single register
People and Remuneration	Board composition and the appointment pipeline, induction and board evaluation, executive and senior remuneration, the workforce position including vacancy and turnover, the conflicts and related-party register, the sitting allowance and participation policy



Committee	Its standing content in sections 2 to 5
Transformation and Digital	Roadmap milestones against plan, system selection and implementation status, data governance and information security posture, benefits realised against the case, and the committee's own sunset review
Afya-HSH Governance Committee	Operational performance against the agreed KPI set, partnership milestones, service issues and their resolution, decisions taken within delegated authority, and matters escalated to the board

6. Instrument 5: management report by exception

Three pages, maximum. Written by the Medical Director with the Afya operational leads.

MANAGEMENT REPORT TO THE BOARD

Period: [quarter]
 Prepared by: [Medical Director] with [Afya operational lead]
 Date:

1. THE THREE THINGS THE BOARD SHOULD KNOW
 Three bullets. If a director reads nothing else, this is what they must not miss. At least one should be uncomfortable.
2. PERFORMANCE AGAINST PLAN
 Reference the dashboard. Narrate only what is off plan.
 For each exception, the four standing questions answered:
 - Why is it different from plan?
 - Is it one-off, or a trend?
 - What is being done, by whom, by when?
 - What would make it worse?
3. WHAT WENT WELL, BRIEFLY
 Two or three. Boards that only ever hear problems stop being told about them.
4. WHAT IS COMING
 The next quarter: what management expects, and what could knock it off course.
5. WHERE WE NEED THE BOARD
 Decisions requested (cross-referenced to board papers), steers wanted, and any place management feels blocked.
6. DECLARED ESCALATIONS SINCE THE LAST MEETING
 What crossed a threshold, when it was escalated, to whom, and how it was resolved. Nil returns are stated as nil.

Section 6 is the honesty test of the whole system. If serious things happened and this section is empty, the escalation thresholds are not being used, and that is a governance failure whatever the underlying result looked like.

7. Instrument 6: the performance dashboard and KPI pack



Two pages, produced monthly for the Afya-HSH Governance Committee and consolidated quarterly for the board. Every metric carries **actual, plan, prior period, and a direction of travel**, with a RAG status against a threshold the board has set.

7.1 Activity

Admissions; bed occupancy rate; average length of stay; theatre cases; theatre utilisation; outpatient attendances, new and follow-up; emergency attendances; imaging and laboratory volumes; deliveries where relevant.

7.2 Clinical quality and safety

Crude mortality and reviewed mortality; never events; serious incidents reported and reviewed within policy; healthcare-associated infection rate; unplanned readmission within 28 days; unplanned return to theatre; medication errors; patient falls; surgical safety checklist compliance; complaints received, upheld, and time to resolution; patient experience score.

7.3 Workforce

Vacancy rate by staff group; nurse staffing against establishment; turnover and stability index; sickness absence; overtime and locum spend against budget; credentialing and practising licence currency; mandatory training compliance.

7.4 Finance

Revenue against budget; payer mix across self-pay, HMO, corporate, and retainer; EBITDA against budget; cash days on hand; debtor days, with HMO receivables shown separately and aged; claims submitted, rejected, and the rejection reason profile; collection rate; payroll as a percentage of revenue; drugs and consumables as a percentage of revenue; capital spend against plan.

7.5 Access and experience

Outpatient waiting time; time from decision to theatre; emergency door-to-clinician time; discharge before noon; cancelled operations and the reason.

7.6 Partnership and transformation

Afya milestones against plan; the agreed partnership KPI set; management fee and any service credits; transformation roadmap milestones; benefits realised against the business case.

7.7 Risk and compliance

Open high-rated risks; overdue risk actions; overdue incident reviews; regulatory and accreditation status; statutory filings status; open litigation and claims.

A note on discipline. The board sees the dashboard, not the source data. Three pages of charts a director cannot interrogate is not oversight. If a metric has never once prompted a question in four meetings, either the threshold is wrong or the metric should not be on the board's page.

To be set by the board at the September meeting: the RAG threshold for each metric. Until thresholds are agreed, the dashboard reports the numbers without a status colour.

8. Instrument 7: risk register summary



The board sees a summary. The full enterprise register is owned by the Audit and Risk Committee, with the clinical lens read at Quality and Clinical Governance. **One register, two lenses.**

RISK REGISTER SUMMARY TO THE BOARD

As at: [date] Prepared by: [ARC chair / risk owner]
 Total risks: [n] High (15+): [n] Movements this quarter: [n]

TOP RISKS

Ref	Risk	Likeli- hood	Impact	Score	Move- ment	Owner	Target score	Next review
R01	[description]	4	5	20	new	[name]	10	[date]

FOR EACH TOP RISK

What it is, in one sentence a non-specialist understands.
 The controls in place now.
 What is being added, by whom, by when.
 What the board is being asked to do, if anything.

NEW RISKS THIS QUARTER

RISKS CLOSED THIS QUARTER, AND WHY

OVERDUE RISK ACTIONS, WITH OWNER AND REASON

RISK APPETITE: where we are operating outside the appetite the board has set, and whether that is deliberate.

Scoring is 5 by 5, likelihood by impact. **A score of 15 or above escalates to the board automatically**, without waiting to be selected for the pack.

9. Instrument 8: minutes template

Minutes are a statutory record, not a transcript. They exist to show that the board applied its mind. Barr. Otuogha's module made the legal status plain; this is the practical shape.



HAVANA SPECIALIST HOSPITAL LIMITED
 MINUTES OF THE [BOARD / COMMITTEE] MEETING
 HELD ON [date] AT [time], AT [venue and video]

PRESENT

[Name, role] [in person / video]

IN ATTENDANCE

[Name, role, and for which items]

APOLOGIES

[Name, role]

QUORUM

The Chair confirmed that a quorum was present and the meeting was properly constituted, notice having been given in accordance with the articles.

1. DECLARATIONS OF INTEREST

[Name] declared an interest in item [n] as [nature of interest].
 [Name] withdrew from the meeting for item [n] and took no part in the discussion or the decision. The register was updated.

2. MINUTES OF THE PREVIOUS MEETING

RESOLVED: that the minutes of the meeting held on [date] be approved as a correct record and signed by the Chair.

3. [SUBSTANTIVE ITEM]

The board received paper [ref] from [sponsor].
 The board noted [the key facts relied on].
 The board discussed [the substance, including the principal points of challenge and any dissent, without attributing every remark].
 [Where a director dissented and asked for it to be recorded, record it by name.]
 RESOLVED: that [the decision, in operative words], for the reasons that [the reasons, in one or two lines].
 ACTION: [owner] to [action] by [date].

...

[N]. IN-CAMERA SESSION

The non-executive directors met without the executives present.
 [Chair to note any matter arising, or that none arose.]

[N]. DATE OF NEXT MEETING

There being no further business the Chair closed the meeting at [time].

Signed: _____ Chair Date: _____

What good minutes do and do not do.

- **Do** record the decision in operative words, and the reasons for it.
- **Do** record the information the board relied on, so a later reader can see the board was properly informed.
- **Do** record declarations, recusals, and withdrawals precisely, including that the director took no part in the decision.
- **Do** record dissent where a director asks for it. That is the director's protection.
- **Do not** attribute every contribution by name. Minutes that read like a transcript make directors cautious, and cautious directors challenge less.
- **Do not** leave a decision implied. If it is not resolved in the minute, it was not decided.
- **Draft within five working days**, to the Chair within seven, and circulate with the next pack.



10. Instrument 9: action and decision log

Rolling. Carried into every meeting, never restarted.

ACTION AND DECISION LOG, [BOARD / COMMITTEE]

Ref	Meeting	Decision or action	Owner	Due	Status	Note
B/26/09/03	24 Sep	Approve dialysis business case for December board	MD	Dec 26	Open	Paper due

STATUS: Complete / On track / At risk / Overdue / Deferred (with the meeting that agreed the deferral)

RULES

- An action has one named owner. Not a committee, not "management".
- An action has a date. "Ongoing" is not a date.
- Overdue actions are listed at every meeting until closed. They do not quietly disappear.
- Decisions stay on the log for one meeting after completion so the board sees the decision closed out, then move to the decision register held by the Company Secretary.

11. Instrument 10: escalation thresholds schedule

What must reach the board or a committee chair **between meetings**, without waiting for the quarter and without anyone having to judge the moment. These are proposed defaults for the board to set at the September meeting.

Trigger	Escalates to	Timeframe
Never event, or a death in unexpected circumstances	Board Chair and Quality and Clinical Governance chair, then all directors	Same day
Serious clinical incident, or a cluster of related incidents	Medical Director to Quality and Clinical Governance chair	24 hours
Regulatory inspection, notice, sanction, or licence issue	Board Chair, Audit and Risk chair	24 hours
Litigation, or a claim above [N5m, to be set]	Board Chair, Audit and Risk chair	48 hours
Data or information security breach	Board Chair, Transformation chair, Audit and Risk chair, and the data protection lead for statutory notification	24 hours internally, statutory clock runs separately
Cash below [30 days, to be set] of operating cost	Board Chair, Finance co-chairs	Immediate
Budget variance beyond [10 per cent on a line, or 5 per cent at EBITDA]	Finance, Investment and Operations co-chairs	Next monthly cycle, or immediate if material
Any risk newly rated 15 or above	Audit and Risk chair, then the board	Within the week
Unbudgeted commitment above [the delegation threshold]	Board, as a reserved matter	Before commitment



Trigger	Escalates to	Timeframe
Loss of, or material change to, a significant HMO or corporate contract	Board Chair, Finance co-chairs	48 hours
Departure or suspension of a senior clinician or executive	Board Chair, People and Remuneration chair	Same day
Any proposed change to the Afya agreement	Board, as a reserved matter	Before agreement
Media, reputational, or community event of significance	Board Chair	Same day

The rule that makes this work. Escalating is never penalised. A threshold crossed and escalated is the system working. A threshold crossed and not escalated is the only version of this that is a disciplinary matter. Say that out loud when the board adopts the schedule.

12. Instrument 11: register of interests and attendance

12.1 Register of interests

Maintained continuously by the Company Secretary, declared afresh at the start of each board year, and updated within seven days of any change.

Field	
Director	
Nature of interest	Directorship, shareholding, employment, family relationship, supplier relationship, professional appointment
Entity	
Date declared	
Related to HSH how	
Management applied	Recusal, non-participation, information withheld, none required

Known standing entries at adoption include the family directors' shareholder interests, the Afya relationship, and Consult for Africa's adviser relationship, on which Dr Odulana recuses from any decision on its fees or scope.

12.2 Attendance register

Published annually to the board, and to the People and Remuneration Committee, which owns the participation policy.

Director	Board (of 4)	Committee (of n)	Papers read on time	Notes
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Attendance is the floor, not the standard. The People and Remuneration Committee reviews it alongside the sitting allowance and participation policy.



13. The annual board and committee calendar

Rolling twelve months, sequenced so committees always feed the board rather than trailing it. Dates are indicative and are confirmed by the Company Secretary; the **pattern** is what the board is adopting.

The sequencing rule. Committees meet **two to three weeks before** the board. The pack is circulated **seven clear days** before. The Afya-HSH Governance Committee meets monthly and its report for the quarter closes before the committee week.

13.1 Establishment phase, September to December 2026

The operating model allows a tighter cadence for the first two or three meetings, then a settle to quarterly. This uses it.

Date	Meeting
Tue 1 Sep 2026	Afya-HSH Governance Committee, monthly
Thu 3 Sep 2026	Transformation and Digital Committee
Tue 8 Sep 2026	Finance, Investment and Operations Committee
Wed 9 Sep 2026	Audit and Risk Committee, including external audit planning
Thu 10 Sep 2026	Quality and Clinical Governance Committee
Tue 15 Sep 2026	People and Remuneration Committee
Thu 17 Sep 2026	Board pack circulated
Thu 24 Sep 2026	BOARD MEETING , adoption of this instrument suite
Tue 6 Oct 2026	Afya-HSH Governance Committee
Thu 8 Oct 2026	Transformation and Digital Committee
Mon 19 to Thu 22 Oct 2026	Committee week, all five
Thu 29 Oct 2026	Board pack circulated
Thu 5 Nov 2026	BOARD MEETING , six-weekly establishment cadence
Tue 3 Nov, Tue 1 Dec 2026	Afya-HSH Governance Committee
Tue 24 to Thu 26 Nov 2026	Committee week
Thu 3 Dec 2026	Board pack circulated
Thu 10 Dec 2026	BOARD MEETING , year-end, budget approval for 2027

13.2 Settled quarterly cycle, 2027

Date	Meeting
Fri 29 to Sat 30 Jan 2027	Annual strategy session , board and senior management
Monthly, first Tuesday	Afya-HSH Governance Committee
Tue 2 to Thu 4 Mar 2027	Committee week



Date	Meeting
Thu 11 Mar 2027	Board pack circulated
Thu 18 Mar 2027	BOARD MEETING , Q1, annual financial statements and going concern
Tue 1 to Thu 3 Jun 2027	Committee week
Thu 17 Jun 2027	BOARD MEETING , Q2, board evaluation and Chair review
Tue 31 Aug to Thu 2 Sep 2027	Committee week
Thu 16 Sep 2027	BOARD MEETING , Q3
Tue 23 to Thu 25 Nov 2027	Committee week
Thu 9 Dec 2027	BOARD MEETING , Q4, budget approval for 2028

13.3 Fixed points in the governance year

When	What	Owner
Q1 board	Annual financial statements, external audit report, going-concern judgement	Audit and Risk
Q1 board	Confirmation of the Chair's interim appointment, per the operating model	People and Remuneration
Q2 board	Board evaluation and director self-assessment	People and Remuneration
Q2 or Q3	Annual general meeting , and annual return to the Corporate Affairs Commission	Company Secretary
Q3 board	Risk appetite review, insurance and directors' indemnity renewal	Audit and Risk
Q4 board	Annual budget and capital plan approval	Finance, Investment and Operations
Q4 board	Sunset review of the Transformation and Digital Committee	Board
Annually	Register of interests refreshed, declarations retaken	Company Secretary
Annually	Terms of reference and delegation thresholds reviewed	People and Remuneration

To confirm before adoption. The AGM date and the annual return deadline follow HSH's financial year end and its incorporation date, which the Company Secretary should confirm and place in the calendar. The legal faculty confirms the statutory deadlines; this pack sets the governance rhythm around them.



14. What the board is asked to do in September

The pack is not adopted by being circulated. Six decisions turn it into the way HSH runs.

1. **Adopt the instrument suite**, instruments 1 to 11, with effect from the September meeting.
2. **Set the escalation thresholds** at section 11, filling the bracketed values.
3. **Set the RAG thresholds** for the dashboard metrics at section 7.
4. **Adopt the calendar** at section 13, and confirm the establishment-phase cadence of six-weekly for the first three meetings.
5. **Confirm the delegation thresholds** in the Delegation of Authority schedule, which the escalation schedule depends on.
6. **Instruct the Company Secretary** to administer the suite, maintain the registers, and report compliance with the seven-day pack rule at each meeting.

Once those six are minuted, the governance framework is not a document. It is the operating reality.

Prepared by Dr Debo Odulana for the Board of Havana Specialist Hospital. Consult for Africa. Confidential. This pack sets governance process and is not legal advice; statutory deadlines, filing obligations, and the requirements of the articles are confirmed by the company's legal advisers and the Company Secretary.